Competitive Landscape: non-small cell lung cancer (EU, 2024)

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Market Overview

In the EU non-small cell lung cancer market, 4 brands compete for second-line+ patients as of Q3 2024. The segment is shaped by guideline updates, reimbursement decisions and real-world evidence.

The number of branded options in non-small cell lung cancer has grown since 2022, increasing pressure on standard of care positioning.

Positioning: Keytrava

In the EU non-small cell lung cancer market, Keytrava maintains a 15% market share as of Q2 2024, with the strongest uptake among oncologists.

Sentiment index for Keytrava in EU: 0.38 (scale 0 to 1, 2024). Clinicians view the brand negatively; cost and formulary access remain barriers.

Positioning: Opdivra

In the EU non-small cell lung cancer market, Opdivra maintains a 25% market share as of Q4 2024, with the strongest uptake among oncologists.

Sentiment index for Opdivra in EU: 0.29 (scale 0 to 1, 2024). Clinicians view the brand negatively; payer prior-authorisation requirements slow initiation.

Positioning: Tagrisa

In the EU non-small cell lung cancer market, Tagrisa maintains a 22% market share as of Q3 2024, with the strongest uptake among oncologists.

Sentiment index for Tagrisa in EU: 0.74 (scale 0 to 1, 2024). Clinicians view the brand strongly positively; long-term safety data are still limited.

Positioning: Lumakro

In the EU non-small cell lung cancer market, Lumakro maintains a 35% market share as of Q4 2024, with the strongest uptake among oncologists.

Sentiment index for Lumakro in EU: 0.60 (scale 0 to 1, 2024). Clinicians view the brand moderately positively; long-term safety data are still limited.

Competitive Dynamics

Lumakro has been gaining ground since its 2021 approval for non-small cell lung cancer, pressuring Keytrava in first-line use.

Price competition, biosimilar entry and head-to-head evidence are expected to reshape formulary position over the next 24 months.

Dose modification guidance for Keytrava is generally followed, but oncologists said that practical monitoring of infusion reactions depends on local capacity and patient travel distance.

Registry data show that outcomes in non-small cell lung cancer depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Respondents expect real-world persistence on Keytrava to be lower than in trials, mainly because of missed maintenance doses and the complexity of managing fatigue.

Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Treatment burden, including monitoring visits and missed maintenance doses, was described as a key factor in long-term adherence and persistence.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Clinical guidelines in Spain recommend first-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Outlook

We expect continued consolidation in non-small cell lung cancer. Differentiation will depend on tolerability, dosing convenience and evidence of overall survival or clinical benefit.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Subgroup analyses suggest that benefit with Keytrava is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

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Safety monitoring focused on fatigue and infusion reactions, with dose interruption or discontinuation used for severe adverse events.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between PD-1 inhibitor options.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

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Synthetic document generated for the Watson prototype. All brand names and figures are fictional.